

It is estimated that more than 120,000 Americans take their own lives each year. This figure includes cases which are officially recorded as suicides and the cases of those who do away with themselves, but whose deaths, for one

270

reason or another, are not recorded officially as such. A significant portion of these 120,000 annual tragedies are classed as "economic suicides."

According to Dr. Thomas P. Malone, head of the Atlanta, Georgia, Psychiatric Clinic, an acknowledged authority on the macabre subject: "At least 30 to 40 percent of so-called economic suicides occur when a man is successful, not when he is failing. When a man has achieved the peak of success, often he has nothing left to scramble for."

I'm no psychiatrist, but it seems to me that anyone who takes his own life because he has achieved success and has

"nothing left to scramble for" never had any worthwhile motives to scramble for in the first place. The goals he sought—and achieved—were meaningless. When he realized this he also realized that what he had actually achieved was not success but pathetic failure.

In a report in the *Journal of the American*

*Medical Association*, Drs. Richard E. and Katherine K.

Gordon revealed the results of an intensive study they made of families living in a typical contemporary status-seekers' suburban community. They determined that the diseases which stem primarily from emotional stresses

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notably ulcers, coronary thrombosis, hypertension and hypertensive cardiovascular disease—were markedly more prevalent there than in communities in which status seeking was not such a dominant social factor. Anyone who has encountered specimens of the ulcer-ridden, tranquilizer-